

VALIDATION OF EOLIFE X

REGULAR 581 VENTILATIONS

VT_i
inspiratory
tidal volume

BIAS **WITHIN** CLINICALLY
ACCEPTABLE DIFFERENCE
LOA **WITHIN** CLINICALLY
ACCEPTABLE DIFFERENCE

VT_e
expiratory
tidal volume

BIAS **WITHIN** CLINICALLY
ACCEPTABLE DIFFERENCE
LOA **WITHIN** CLINICALLY
ACCEPTABLE DIFFERENCE

f
ventilation
rate

BIAS **WITHIN** CLINICALLY
ACCEPTABLE DIFFERENCE
LOA **WITHIN** CLINICALLY
ACCEPTABLE DIFFERENCE

INTRA-ARREST 240 VENTILATIONS (ASYNCHRONOUS)

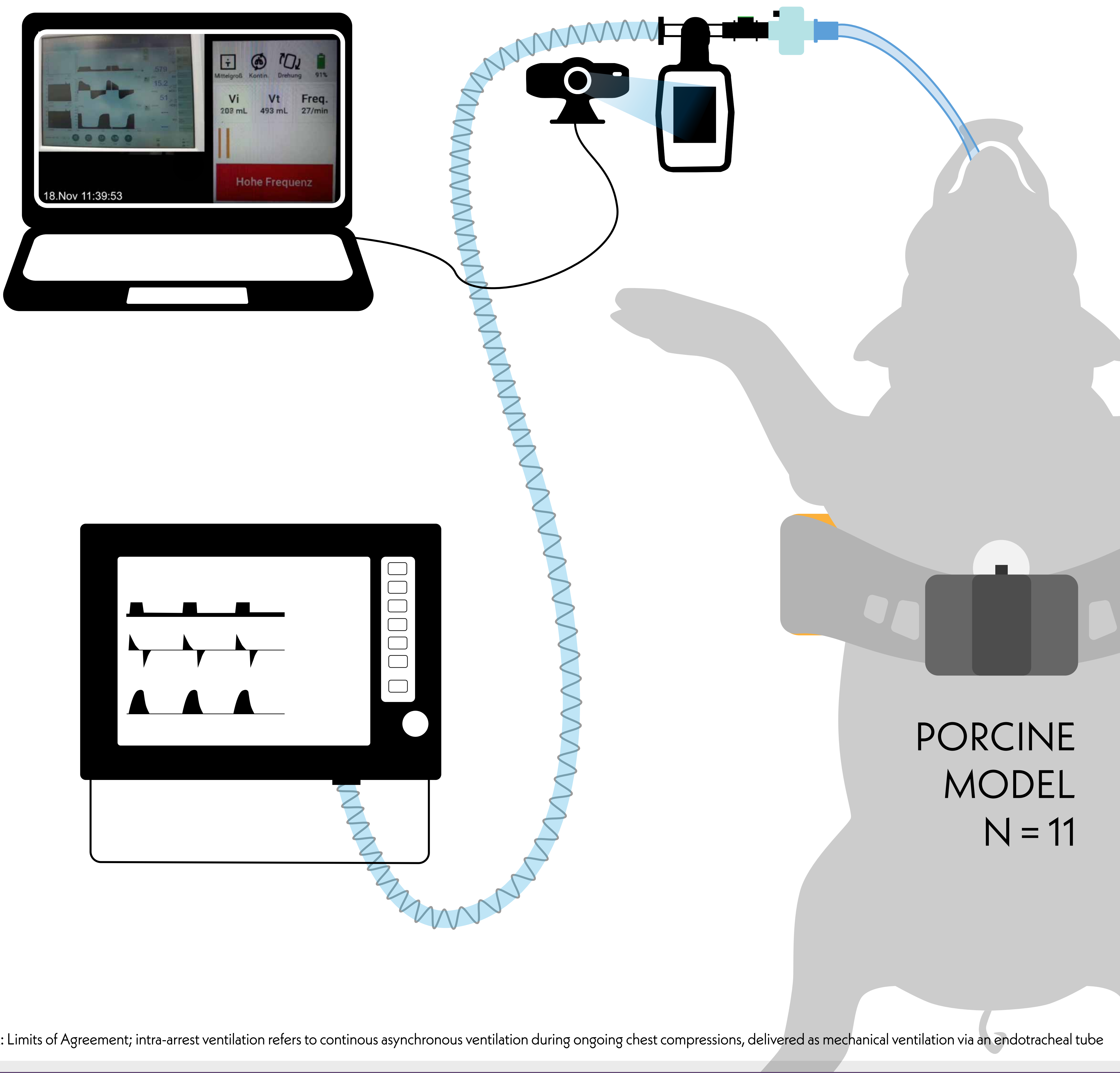
BIAS **WITHIN** CLINICALLY
ACCEPTABLE DIFFERENCE **-26ML**
LOA **OUTSIDE** CLINICALLY
ACCEPTABLE DIFFERENCE **-355 TO +304ML**

BIAS **OUTSIDE** CLINICALLY
ACCEPTABLE DIFFERENCE **-281ML**
LOA **OUTSIDE** CLINICALLY
ACCEPTABLE DIFFERENCE **-599 TO +37 ML**

BIAS **OUTSIDE** CLINICALLY
ACCEPTABLE DIFFERENCE **+23.8/MIN**
LOA **OUTSIDE** CLINICALLY
ACCEPTABLE DIFFERENCE **-50.2 TO +97.8/MIN**

POSSIBLE
MECHANISM:

REVERSE AIRFLOW EPISODES
COMPLICATE MEASUREMENTS



PORCINE
MODEL
N = 11

LoA: Limits of Agreement; intra-arrest ventilation refers to continuous asynchronous ventilation during ongoing chest compressions, delivered as mechanical ventilation via an endotracheal tube

